INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Continuous fluorouracil + radiotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
7 days
See the active treatment phase

Treatment stages
2
Explained in the treatment reference

Regimen source
NCCP 00421
Version 6

> Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.

Treatment stages

01 **Prescribed treatment course**
Locally advanced rectal cancer

02 **Radiotherapy plan**
Separate appointments

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Contact oncology immediately for fever, shivering, sudden illness, severe diarrhoea, inability to drink, very little urine or unusual bleeding. New severe chest pain, collapse or major breathing difficulty needs emergency help.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

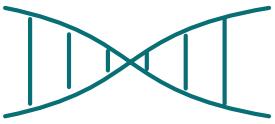
Your medicines and what they do

Continuous fluorouracil helps radiotherapy treat locally advanced rectal cancer. Your team explains how this fits with surgery and other treatment.

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with deoxyribonucleic acid (the cell's genetic material) (DNA) and RNA, the instructions cells need to grow. The same drug has different schedules; follow the prescription for this regimen.

Drug enters cell → DNA / RNA disrupted → Growth slows

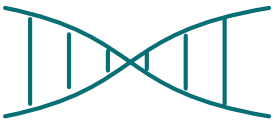
What to watch for

Report diarrhoea, mouth sores, painful hands or feet and unexpected severe early symptoms. Chest pain needs urgent assessment even after a normal dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) test.

LOCAL RADIATION TREATMENT

Pelvic radiotherapy

External beam treatment



Targets the pelvic treatment area

Radiation is directed at the planned area. Fluorouracil helps sensitise cancer cells to it. Nearby bowel, bladder and skin may also become irritated. Effects can build over the weeks and may continue after the final session.

Planned radiation field → Damage to cancer-cell DNA → Cells lose the ability to multiply

What to watch for

Tell the radiotherapy team about diarrhoea, pain, skin soreness or urinary changes. Discuss possible longer-term bowel, bladder, sexual and fertility effects before treatment.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim and duration

Your course

Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.

Choices

Discuss the expected benefit, burdens, alternatives and how treatment fits your priorities. You can ask for more time or another explanation before deciding.

Understanding the evidence

Ask which evidence matches your treatment aim.

The source supports continuous fluorouracil with radiotherapy for locally advanced rectal cancer. Radiation technique, later surgery and other systemic therapy affect outcomes; no survival estimate from a different schedule is applied.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Your treatment plan

Treatment stages

01 **Prescribed treatment course**
Locally advanced rectal cancer

02 **Radiotherapy plan**
Separate appointments

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

LOCALLY ADVANCED RECTAL CANCER

Prescribed treatment course

Start | Treatment | Review

Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.

SEPARATE APPOINTMENTS

Radiotherapy plan

Planning | Radiation | Recovery

The radiation team confirms the field, fractions, appointments and aftercare; pump days do not define radiation attendance.

The treating team confirms the indication, schedule and duration. Changes are recorded on your own prescription.

BASELINE → TREATMENT → FOLLOW-UP

Tests and checks

Required safety blood tests are separate from the individually prescribed scan or tumour-marker plan.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00421 v6 official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN
For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Continuous fluorouracil + radiotherapy: treatment and checks

Locally advanced rectal cancer

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Treatment visits	Between visits	Review	After treatment
Blood count / kidneys / liver NCCP 00421	Baseline tests and DPD status	Weekly throughout treatment	Extra tests if unwell or dehydrated	Results guide any delay or change	Follow unresolved abnormalities
Symptoms and medicines NCCP 00421	Review medicines, cardiac history and usual function	Review bowel, mouth, skin and infection symptoms	Use urgent contact plan	Confirm recovery and next dose	Agree recovery care
Scans / tumour markers NCCP 00421	Confirm disease assessment	According to individual plan	New symptoms may bring review forward	Consultant-directed response assessment	Individual follow-up
Radiotherapy / pump NCCP 00421	Radiation planning; line and pump training	Weekly bloods and pump changes; separate radiation visits	Report bowel, bladder, skin or line problems	Assess nutrition and hydration	Radiation/ colorectal team decides response assessment and next steps

What each check is for

1 Confirm your personal plan

Before starting

Continuous fluorouracil helps radiotherapy treat locally advanced rectal cancer. Your team explains how this fits with surgery and other treatment.

Ask / act: Agree medicines, treatment days, duration and monitoring.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.

1 · Before treatment

Choose and prepare the line

The team explains whether a port or peripherally inserted central catheter (PICC) suits your treatment. You receive instructions for the insertion appointment.



2 · Insertion and recovery

A separate appointment may be needed

For a port, numbing medicine is used and a small device is placed under the skin. Ask about sedation, transport and wound care.

3 · Day 1

The nurse connects your pump

After the day-ward medicines, the nurse checks and connects the fluorouracil pump. Ask for a demonstration before leaving.



4 · At home

Continuous infusion throughout the week

Fluorouracil runs continuously, seven days a week, during the six-week radiotherapy course. Keep the pump and dressing protected as taught.

5 · Every seven days

Professional pump change

The team changes the pump weekly and checks the line. Attend the separate radiotherapy schedule. Do not disconnect or change the pump yourself.

6 · Throughout the course

Ongoing line care

The access device stays in place as planned. Keep the written line-care and urgent-contact instructions; the team confirms completion and disconnection.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Before treatment

Confirm the treatment aim, duration, DPD result, blood tests, prescription and contact numbers.

STEP 2

Visits and access

Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar. Confirm line access, pump changes and disconnection arrangements.

STEP 3

If treatment is delayed

Ask for revised written treatment and support-medicine instructions. Do not make up missed doses or carry an old plan into a new phase.

STEP 4

Getting help

Keep the wallet card and medicine list available. Contact the team promptly for infection, dehydration, chest symptoms or significant toxicity.

Tests and safety

DPD before fluorouracil

Low DPD activity can cause severe toxicity. Testing helps guide the plan but cannot prevent every reaction. Unexpected severe early symptoms need urgent advice.

Blood tests

Weekly throughout treatment: blood count, renal and liver profile; baseline DPD. Scans/markers follow the consultant plan.

Medicines and interactions

Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Treatment pattern	1 — 7 **Days 1–7** Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.
Symptoms and infection	1 — 7 **Days 1–7** Report on any day; timing varies

The written plan comes first: Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar.

Symptoms can continue: Contact oncology for fever, sudden illness, diarrhoea, mouth soreness or inability to drink, during treatment or the break.

Why these bands stay broad: Bars show planned treatment and possible symptom patterns, not measured risk. Side effects can overlap and persist. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00421 v6 official regimen ↗

There are no guaranteed "safe days". Act on fever, sudden illness or a new serious symptom whenever it occurs. Do not wait for the next appointment.

Between-treatment notes

Day 1: Weekly review and pump change

Check blood tests, line care and the next pump appointment. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Do not change pump settings or disconnect it yourself unless specifically trained and instructed.

Day 2–7: Continuous treatment, including weekends

Keep the pump connected as instructed; radiation appointments follow their own calendar. Side effects can occur on any day; the calendar does not predict when you will feel well. Call the oncology team if unwell. Do not change pump settings or disconnect it yourself unless specifically trained and instructed.

Support around the medicines

1 **Supportive medicines**

Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.

2 **Mouth, bowel and skin care**

Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.

3 **Medicine checks**

4 **Pump and line support**

Confirm the pump-care instructions and a working contact number. Do not adjust, reconnect or disconnect the pump unless specifically trained and instructed.

Recognise symptoms and act

Mouth / eating

Report painful ulcers or difficulty eating and drinking.

Chest / heart

New chest pain needs urgent assessment, including during an infusion.

Bowel / hydration

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

Hands / feet

Report painful redness, cracking or peeling that affects daily activity.

Blood / infection

Fever, shivers or sudden illness need immediate oncology advice.

Pump / line

Report leakage, line pain, swelling, disconnection or a stopped pump immediately.

Pelvic skin / bladder

Report skin pain, urinary symptoms or bleeding; effects can continue after treatment.

Recovery / daily life

Tell the team if tiredness, pain or poor intake limits normal activities.

Daily life and support

Food and fluids

Aim for tolerable meals and fluids. Report weight loss or poor intake; ask for dietetic support.

Activity and rest

Adjust activity and rest to your energy. Report fatigue that limits normal tasks.

Fertility and sexual health

Ask about fertility preservation before treatment and the contraception and breastfeeding plan for all medicines. Report a possible pregnancy promptly.

Support for you and your family

Ask about counselling, psycho-oncology, practical support, transport and financial advice. Bring someone to discussions if helpful.

Radiotherapy and recovery

Separate appointments

Follow the radiation team's preparation and appointment calendar. Seven-day infusion does not mean seven radiation visits per week.

Pump throughout the week

The fluorouracil pump runs continuously, including weekends, and is changed every seven days. A radiation interruption requires an explicit chemotherapy plan.

After treatment

Bowel, bladder, skin and fatigue effects may persist or develop later. Agree follow-up and the next colorectal treatment decision.

Your home infusion

Before leaving

Check the exact finish or change time, disconnection appointment, line care and 24-hour contact route.

If something goes wrong

Report leakage, swelling, line pain, disconnection, an alarm or unexpected pump speed. Follow the supplied spill instructions; do not handle leaked medicine with bare hands.

Changes to treatment

Do not restart, reconnect or change the pump unless specifically trained and instructed. The team updates the prescription and pump plan.

Questions worth asking

Which treatment plan applies to me?

Fluorouracil runs continuously through a pump, seven days a week, during a six-week radiotherapy course. The pump is changed every seven days. Radiotherapy has its own appointment calendar. Your team confirms any modification.

Do more side effects mean treatment is working?

No. Side effects do not reliably measure response. Report symptoms and follow the agreed review plan.

Who do I contact overnight?

Ask the team to replace example numbers with working daytime, out-of-hours and backup contact routes before leaving.

Try explaining the plan back: Can you explain which medicines you are receiving, the schedule, the contact plan and the symptoms that need prompt help?

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous fluorouracil with radiotherapy

Active medicines: Fluorouracil

Emetogenic risk: Low

Agree an individual low-risk antiemetic/rescue plan. Do not automatically repeat an intravenous (IV) premedication dose on every day of a continuous six-week infusion.

Additional phase-specific support

- Use the prescribed anti-sickness and diarrhoea plan; check the medicine, timing and rescue duration. Do not duplicate steroids or anti-sickness medicines.
- Report soreness and diarrhoea early. Use gentle mouth and skin care and ask for help with food or fluids.
- Bring all prescription medicines, supplements and non-prescription products. Warfarin and phenytoin may need closer monitoring; brivudine/sorivudine interactions require pharmacy review.
- Confirm the pump-care instructions and a working contact number. Do not adjust, reconnect or disconnect the pump unless specifically trained and instructed.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Mouth / eating

Fluorouracil

Report painful ulcers or difficulty eating and drinking.

2 Chest / heart

Fluorouracil

New chest pain needs urgent assessment, including during an infusion.

3 Bowel / hydration

Fluorouracil + pelvic radiotherapy

Report diarrhoea, increased stoma output, severe pain, blood or dehydration.

4 Hands / feet

Fluorouracil

Report painful redness, cracking or peeling that affects daily activity.

5 Blood / infection

Chemotherapy

Fever, shivers or sudden illness need immediate oncology advice.

6 Pump / line

Fluorouracil delivery

Report leakage, line pain, swelling, disconnection or a stopped pump immediately.

7 Pelvic skin / bladder

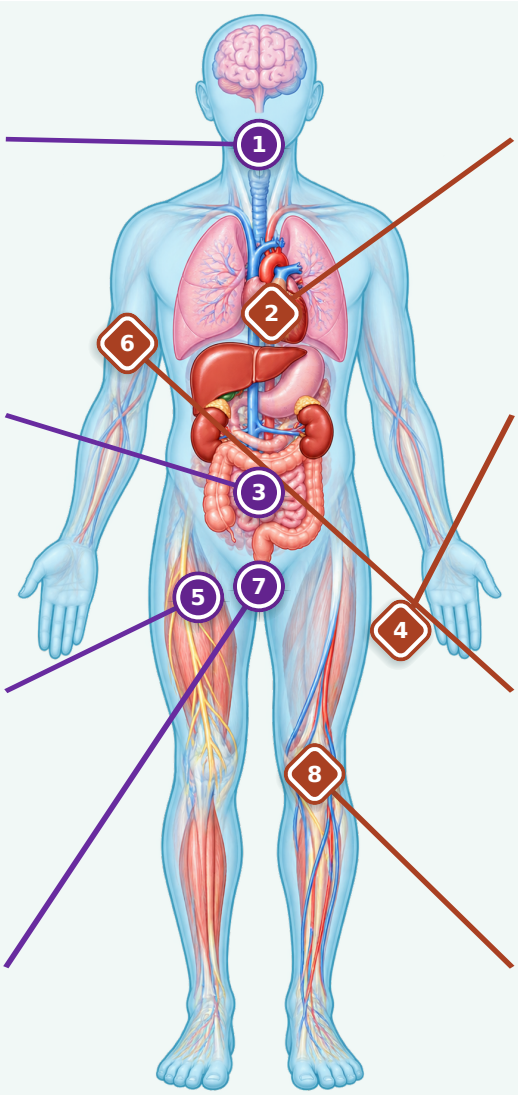
Radiotherapy

Report skin pain, urinary symptoms or bleeding; effects can continue after treatment.

8 Recovery / daily life

Chemoradiotherapy

Tell the team if tiredness, pain or poor intake limits normal activities.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Contact oncology immediately for fever, shivering, sudden illness, severe diarrhoea, inability to drink, very little urine or unusual bleeding. New severe chest pain, collapse or major breathing difficulty needs emergency help.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00421 v6 official regimen**
   https://healthservice.hse.ie/documents/6460/421_5-Fluorouracil_and_radiotherapy (RT).pdf
2. **eviQ: port**
   https://www.eviq.org.au/p/507
3. **eviQ: peripherally inserted central catheter (PICC)**
   https://www.eviq.org.au/p/543
4. **eviQ: pump**
   https://www.eviq.org.au/p/502
5. **NCCP NCIS antiemetic guidance V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
6. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq